17. THE EYE AND THE CORTICALISATION MOVEMENT

DURATION : 08:22

STUDY SHEET

COURSE SUMMARY

This video explores the dynamic link between the eye and the corticalisation movement in brain development. The process of cephalic and cervical flexion is examined, as well as the reorganisation of brain tissues in relation to facial and visceral structures. The eye is presented as a central player in this movement, influencing the formation of the falx cerebri and the tentorium cerebelli, while integrating psychic and emotional aspects through its link with the hormonal system. The concepts of the diaphragm of equilibrium and the terminal fulcrum are introduced, highlighting the importance of stabilising the cruciform suture to rebalance the eyes and promote bodily harmony. This lesson offers practical and theoretical keys for practitioners wishing to better understand embryological development and its therapeutic implications.

THE EYE AND THE CORTICALISATION MOVEMENT

The **brain**, initially perceived as a flat tissue, undergoes a significant transformation during its development. Imagine an individual lying on a table, with their brain in hand. The first piece of information to remember is that of **expansion** and **neural tube formation**.

This process begins with a **cephalic flexion movement**, followed by **cervical flexion**, then **pontine** development and a **telencephalic** process. As the brain straightens, the smaller structures follow this developmental movement, indicating a change in orientation.

The **eye**, in its developmental spiral, contains the information of **corticalisation**. During this straightening, all the tissue reorganises in a rotational movement, moving closer to the **midline**. Below, the tissue pulls towards the heart and digestive system, while above, it straightens. This movement is created by the **ascension of the brain** and the **descent of the cardio-visceral**.

The eye plays a crucial role in this phenomenon of **corticalisation**, contributing to the development of the **falx cerebri** and the **tentorium cerebelli**, which inserts onto the clinoid processes. Nearby is the **optic chiasm**, an essential element to consider.

Another important aspect is the **diaphragm of balance** and **Tenon's capsule**, which connects the eye to the entire facial system and all the body's fascia. This means that the eye constitutes an integral **unit**, containing all facial information. It also receives emotional and psychic information, conveyed by hormonal substances. Thus, the psychic state is intrinsically linked to a hormonal and emotional state.

Treating the eye therefore means addressing **peritoneal**, **peripheral**, and **cortical** development simultaneously. The eye is related to the totality of the body in its expression and development. Each organ has its own point of view, and each organ is a symbol.

Around the **26th day**, an **optic sulcus** appears, linked to an internal connection. This sulcus is stimulated by a vesicle that touches the superficial wall. Cephalic and cervical flexion lead to a meeting between the brain and the heart, with the eye settling on the heart. This creates a **posterior fold**, known as the **pontine flexure**, where the sacrum and the tentorium cerebelli are located.

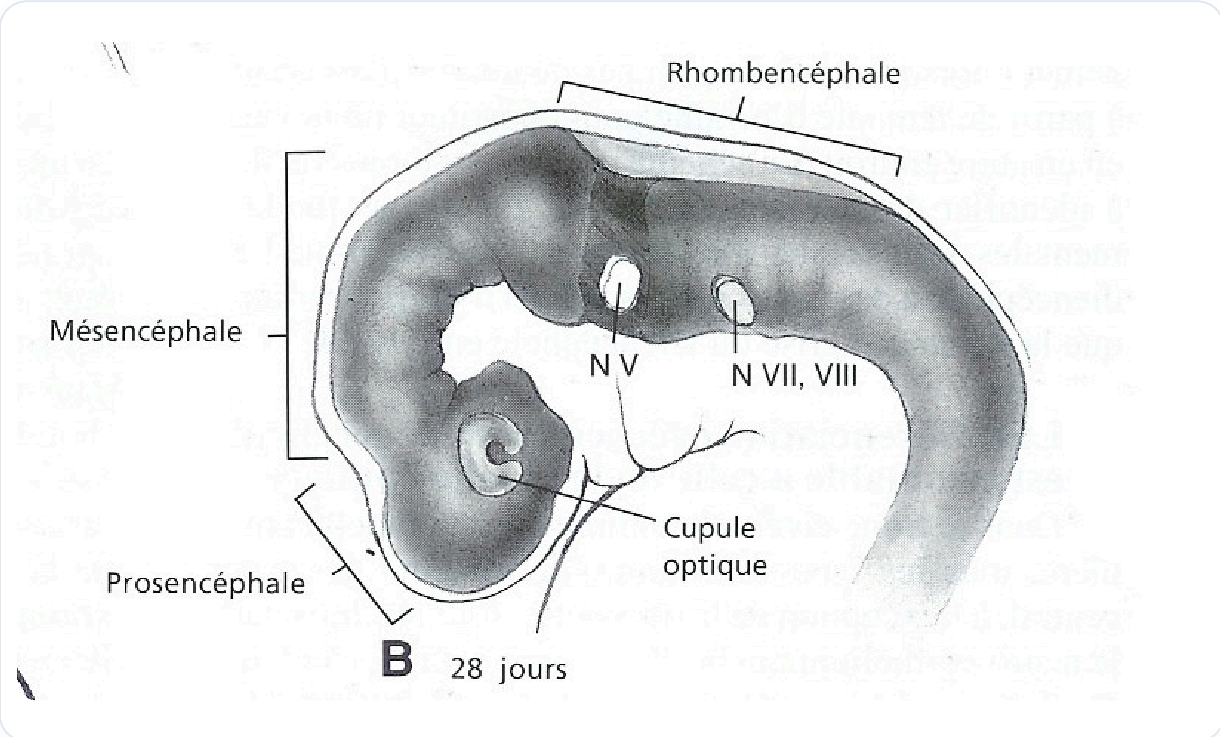

FIG. — Optic sulcus and vesicle

The **tentorium cerebelli** is paramount, as it establishes synchronicity with the peritoneum and the brain. The **submesencephalic flexure** is related to the tip of the **notochord**, serving as a fulcrum. The **supraocciput** is also linked to cervical flexion, while the pontine flexure is associated with the occipital base.

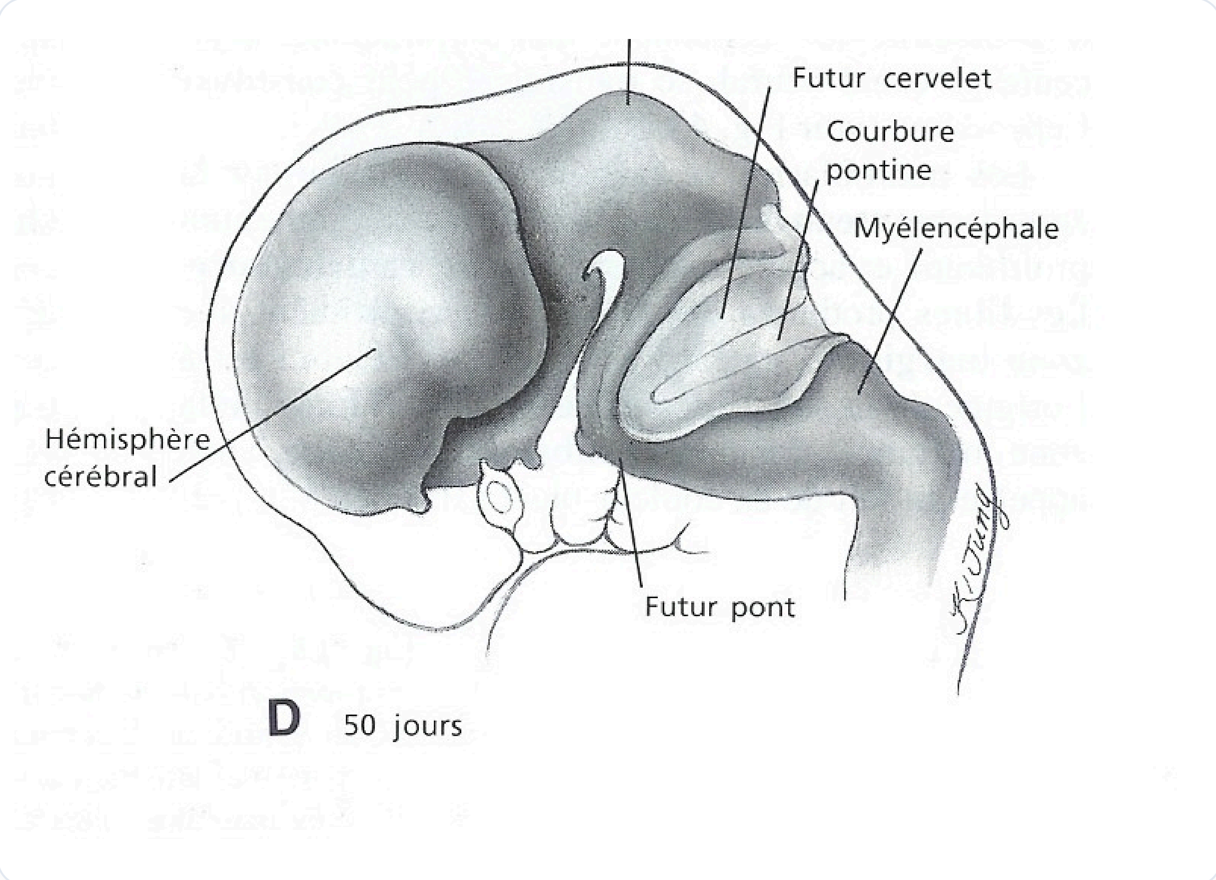

FIG. — Meeting of the brain and heart

The ascension of the neural tube influences the development of the digestive and cardiac systems, as well as other developmental spaces. **Colocalities** and **synchronicities** during morphogenesis help to understand the fulcrums. The ascension of the **ectoderm** and the descent of the **endoderm** are essential for brain integration.

The eye follows this developmental phase, moving closer to the midline to form the **crista galli**, which brings the membranes together to create the falx cerebri. The tentorium cerebelli also develops during the pontine phenomenon.

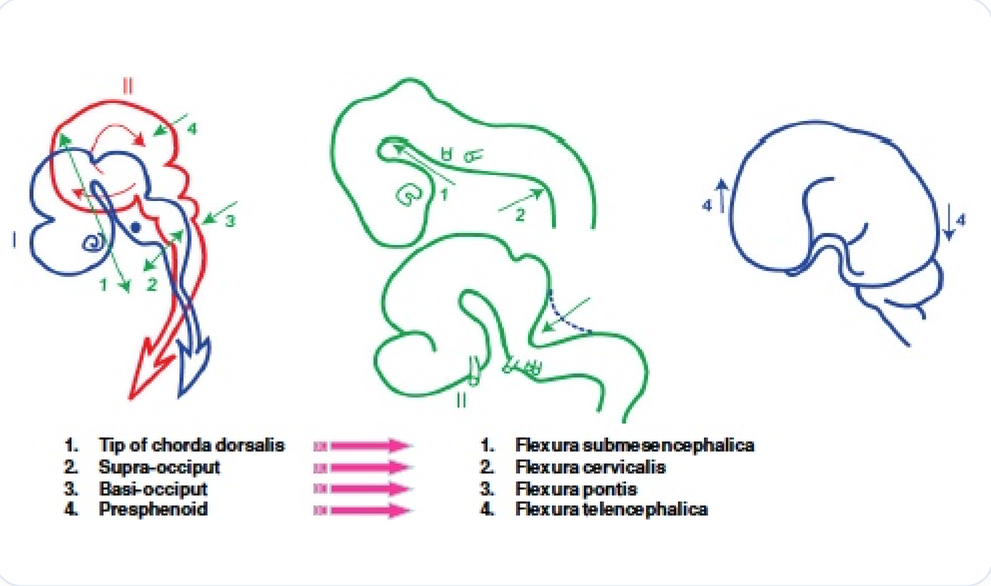

FIG. — Support points

It is crucial to note that the fulcrum, or **terminal fulcrum**, for the placement of the eyes is essential. To rebalance the eyes, it is recommended to place a finger in the mouth, at the level of the small **cruciform suture**. This allows both eyes to find their space when this suture stabilises.

The **frontal margin**, the **zygomatic margin**, and the **maxillary margin** are important fulcrums. The ascent and descent of the structures contribute to the closure of the midline, which is also linked to the closure of the palate. When the palate finds its fulcrum, the eyes also find their place, marking an **arrival fulcrum** in development.